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EASE (Efficacy of ATX01 Study in Erythromelalgia) (EASE)

ClinicalTrials.gov ID ⓘ NCT05917912

Sponsor ⓘ AlgoTherapeutix

Information provided by ⓘ AlgoTherapeutix (Responsible Party)

Last Update Posted ⓘ 2024-04-18

Completed ⓘ

No longer looking for participants

Record History Tab

Study Record Versions

- This table shows all the versions of this study record arranged in order by submitted date.
 - To view one version of the study record, click the submitted date.
 - To compare two versions, select them using the check boxes and click "Compare" at the bottom of the list.
 - Data that is changed in the study record is color coded:
 - Deletions from the study record are shown crossed out in red highlights.
 - Additions to the study record are shown in green highlights.

☐ 1	2023-06-14 None (earliest version on record)
☐ 2	2023-07-25 - Recruitment Status - Study Status - Arms and Interventions - Outcome Measures - Contacts/Locations
☐ 3	2024-01-30 Study Status
☐ 4	2024-04-17 - Recruitment Status - Study Status - Study Design
Compare	

Feedback

	Contacts/Locations
5	2025-03-18 - Study Status - Outcome Measures - Document Section - Participant Flow - Baseline Characteristics - Outcome Measures (Results) - Adverse Events - More Information Quality Control Review Has Not Concluded Returned: 2025-04-02

Viewing **V5** (2025-03-18)

Quality Control Review Has Not Concluded

Note: The results information displayed below has not completed the quality control (QC) review process. ClinicalTrials.gov must post results information for applicable clinical trials (ACTs) within 30 days of submission, even if the submission has not completed the QC review process. The study sponsor or investigator is responsible for ensuring the results information meets the QC review criteria.

This submission includes brief standardized QC review comments added by the National Library of Medicine (NLM). These comments indicate the location of apparent errors, deficiencies, or inconsistencies. For more information, see the [Record Review](https://clinicaltrials.gov/submit-studies/prs-help#record-review) (https://clinicaltrials.gov/submit-studies/prs-help#record-review) section on the PRS Help Resources page.

Study Details

Study Identification

	Date submitted (YYYY-MM-DD) & Changes
Brief Title	EASE (Efficacy of ATX01 Study in Erythromelalgia)
Official Title	A Randomized, Double-blind, Placebo-controlled, 2-period, Crossover Study to Evaluate the Efficacy and Safety of ATX01 (Topical Amitriptyline Hydrochloride 15% w/w) in Adult Patients With Pain Due to Erythromelalgia (EM)
Secondary IDs	2022-003804-34, 161942

Study Status

Record Verification	2025-03
Overall Status	Completed
Study Start	2023-06-14 [Actual]
Primary Completion	2024-02-29 [Actual]
Date submitted (YYYY-MM-DD) & Changes	
First Submitted	2023-06-05
First Submitted that Met QC Criteria	2023-06-14
First Posted	2023-06-26 [Actual]
Results First Submitted	2025-03-18
Last Update Submitted that Met QC Criteria	2024-04-17
Last Update Posted	2025-04-03 [Actual]

Sponsor/Collaborators

Sponsor	AlgoTherapeutix
Responsible Party	Sponsor
Collaborators	

Oversight

U.S. FDA-regulated Drug	Yes
U.S. FDA-regulated Device	No
Data Monitoring	No

Study Description

Brief Summary	The goal of this two-center, randomized, double-blinded, parallel-group, placebo-controlled clinical study is designed to compare the efficacy of twice daily applications of ATX01 versus placebo during two consecutive 3-week treatment periods. The primary objective is the comparison between Treatments (ATX01 15% vs. Placebo) of mean pain attack intensity score assessed for the final week of each treatment period using an 11-point Numerical Pain Rating Scale (NPRS). Mean pain attack intensity is defined as the sum of the pain intensity score of each pain attack during the last 7 full days (Day 14 to Day 20) of each Treatment Period divided by the total number of erythromelalgia pain attacks during that 7-day period. Participants will apply on feet and/or hands twice a day in the morning and in the evening, approximately 12 hours apart from the morning administration for 3 consecutive weeks each and record the pain intensity of each attack that occurs.
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Detailed Description	
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Conditions

Condition	Erythromelalgia
Keywords	

Study Design

Study Type	Interventional
Primary Purpose	Treatment
Study Phase	Phase 2
Interventional Study Model	Crossover Assignment
Number of Arms	2
Masking	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Allocation	Randomized
Enrollment	14 [Actual]

Arms and Interventions

Arms	Assigned Interventions
Active Comparator: Sequence A i)screening and randomization; ii) baseline period (3 weeks); iii) treatment period 1 with placebo(3 weeks); iv) wash-out period (3 weeks); v), treatment period 2 with ATX-01(3 weeks); vi) follow-up visit	Drug: Amitriptyline Hydrochloride - hydrogel bottle of ATX01 15% , 0,5ml (1pump) per hand, 1ml (2 pumps) per feet during 3 weeks Drug: Placebo - hydrogel bottle of placebo , 0,5ml (1pump) per hand, 1ml (2 pumps) per feet during 3 weeks
Active Comparator: Sequence B i)screening and randomization; ii) baseline period (3 weeks); iii) treatment period 1 with ATX-01(3 weeks); iv) wash-out period (3 weeks); v), treatment period 2 with placebo (3 weeks); vi) follow-up visit	Drug: Amitriptyline Hydrochloride - hydrogel bottle of ATX01 15% , 0,5ml (1pump) per hand, 1ml (2 pumps) per feet during 3 weeks Drug: Placebo - hydrogel bottle of placebo , 0,5ml (1pump) per hand, 1ml (2 pumps) per feet during 3 weeks

Outcome Measures

Primary Outcome Measures	1. Comparison Between Treatments (ATX01 15% vs. Placebo) of Mean Pain Attack Intensity Score
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	Comparison between Treatments (ATX01 15% vs. Placebo) of mean pain attack intensity score assessed for the final week of each treatment period using an 11-point Numerical Pain Rating Scale (NPRS). Mean pain attack intensity is defined as the sum of the pain intensity score of each pain attack during the last 7 full days (Day 14 to Day 20) of each Treatment Period divided by the total number of erythromelalgia pain attacks during that 7-day period [Time Frame: during the last 7 full days (Day 14 to Day 20) of each Treatment Period] Quality Control Review Comment provided by the National Library of Medicine: 1. The description of the scale or categories does not include sufficient information to understand the results reported.
Secondary Outcome Measures	1. Mean Pain Attack Intensity Score for Complete Treatment Period 11 point Numeric Pain Rating Scale Scale: 0-10 10 means worst possible pain [Time Frame: through study completion, an average of 15 weeks] 2. Patient Global Impression of Change (PGI-C) Score Patient Global Impression: Global Change (PGI-C) Scale: 1-7 7 means very much worse [Time Frame: through study completion, an average of 15 weeks] 3. Change in Pain Interference With Daily Life Using the Brief Pain Inventory - Short Form Questionnaire (BPI-SF) Brief Pain Inventory Pain Interference Questionnaire Scale: 0-10 10 means completely interferes [Time Frame: through study completion, an average of 15 weeks] 4. Change in Quality of Life, Measured Using the EuroQol-5-Dimension (EQ-5D) Instrument EuroQol-5-Dimension scale [Time Frame: through study completion, an average of 15 weeks] Quality Control Review Comment provided by the National Library of Medicine: 1. The description of the scale or categories does not include sufficient information to understand the results reported. 5. Change in Beck Depression Inventory (BDI) Score. BDI Scale:0-3 3 means worse [Time Frame: through study completion, an average of 15 weeks] 6. Maximum Pain Intensity Score, Assessed by 11-point NPRS. 11 point Numeric Pain Rating Scale Scale: 0-10 10 means worst possible pain [Time Frame: through study completion, an average of 15 weeks] 7. Mean Number of Pain Attacks Per Day [Time Frame: through study completion, an average of 15 weeks] 8. Mean and Maximum Duration of Pain Attacks [Time Frame: through study completion, an average of 15 weeks] 9. Mean Frequency of Use of Rescue Measures (e.g., Cooling). [Time Frame: through study completion, an average of 15 weeks] 10. Mean Duration of Use of Rescue Measures (e.g., Cooling). [Time Frame: through study completion, an average of 15 weeks] 11. Change in Pain Score Before and After Rescue Measures (e.g., Cooling). 11 point Numeric Pain Rating Scale Scale: 0-10 10 means worst possible pain [Time Frame: through study completion, an average of 15 weeks]

Eligibility

Minimum Age	18 Years
Maximum Age	
Sex	All
Gender-based Eligibility	
Gender Eligibility Description	
Accepts Healthy Volunteers	No
Criteria	Inclusion Criteria: 1. Male or female patients 18 years of age and older. 2. Documented diagnosis of erythromelalgia, as characterized by redness, warmth, and burning pain of the extremities (most commonly feet), typically precipitated by heat or exercise and relieved by cooling. 3. Mean pain attack intensity, measured on the 11-point NPRS, of ≥4 and ≤9 at baseline and ≥4 pain attacks per week as documented through eDiary use during the 3 weeks prior to randomization (Day -21 to Day -1). 4. Use of concomitant medications, with the exception of topical agents applied to the hands or feet, are permitted if the dose has been stable for at least 4 weeks preceding the screening visit and is planned to be maintained at the same regimen during the course of the study (prior treatment includes pharmacological and nonpharmacological treatments). 5. Patients having signed a written informed consent prior to any study related procedure. 6. Male patients should agree to use a condom along with another medically acceptable contraceptive method, where applicable according to local guidelines, if he is engaged in sexual activity with a woman of childbearing potential (WOCBP) from the day of the signature of the informed consent and up to 90 days after the end-of-study visit. Male patients should agree not to donate sperm until 90 calendar days after the last dose of study drug. 7. Females must comply with the following in order to be enrolled: 1. WOCBP with negative pregnancy test results can be enrolled only if willing to use an acceptable contraceptive method, i.e. oral contraceptives, patch contraceptives, injection contraceptives, implantable hormonal contraceptives, male condom with intravaginal spermicide, diaphragm or cervical cap with spermicide, vaginal contraceptive ring, intrauterine device or system, surgical sterilization (hysterectomy, bilateral oophorectomy, and/or bilateral salpingectomy), tubal ligation/occlusion, vasectomized partner, or sexual abstinence, if this is the patient's current practice, from at least 14 days prior to the screening visit and throughout the study and for at least 30 days after the completion of the study. 2. Or surgically sterilized for at least 6 months. 3. Or menopausal, for at least 1 year. Exclusion Criteria: 1. Clinical evidence of a pre-existing pain disorder in the extremities resulting from another cause than erythromelalgia, e.g. complex regional pain syndrome (CRPS), diabetic neuropathy, chemotherapy-induced peripheral neuropathy (CIPN). 2. Evidence of skin breakdown, ulcers, papules and > +2 pitting edema of the affected limbs.

- 3. Presence of glaucoma.
- 4. Presence of urinary retention (or significant prostatic hypertrophy at risk of urinary retention).
- 5. History of coronary artery disease.
- 6. History and /or presence of major depressive episode.
- 7. The patient has suicidal risk in the opinion of the investigator based upon clinical interview and the Columbia Suicide-Severity Rating Scale.
- 8. Pregnant or lactating women.
- 9. Abnormality in the 12-lead electrocardiogram (ECG) at screening that in the opinion of the investigator increases the risk of participating in the study, such as a corrected QT Fridericia (QTcF) interval >430 ms for males or >450 ms for females.
- 10. A history of additional risk factors for Torsade de Pointe (e.g. heart failure, hypokalemia, family history of Long QT Syndrome).
- 11. The use of concomitant medications within 24 weeks prior to Day 1 and/or during the study or the equivalent of 5 half-lives that prolong the QT/QTc interval, eg, Class 1 antiarrhythmics (e.g. quinidine, disopyramide, procainamide) and Class 3 antiarrhythmics (e.g. amiodarone, sotalol), antihistamines, antipsychotics known to prolong QT interval, and antimalarials (e.g. mefloquine, quinine), tricyclic antidepressants (e.g. AMT), tetracyclic antidepressants (e.g. maprotiline), cisapride.
- 12. The use of monoamine oxidase inhibitors within 24 weeks (or the equivalent of 5 half-lives) prior to Day 1 and/or during the study.
- 13. The use of opioids within 4 weeks (or the equivalent of 5 half-lives) prior to Day 1 and/or during the study.
- 14. History of illicit drug use or confirmed drugs of abuse at screening. Positive urine drug screen for prescribed medication is allowed at the discretion of the investigator.
- 15. Use of more than 2 analgesics (regardless of the route of administration) from different drug classes (including antidepressants and antiepileptics) on top of study drug.
- 16. Treatment with oral or topical amitriptyline or nortriptyline in the past 4 weeks or current treatment with any other tricyclic antidepressant.
- 17. Any known hypersensitivity to amitriptyline (regardless of the route of administration) in any salt form or to any constituent of the topical formulation.
- 18. Use of glutathione, vitamin E, minocycline, or calcium magnesium supplements within 12 weeks of screening.
- 19. Any topical treatment on treated extremities for any indication, other than cosmetic use of creams and lotions, within the 12 weeks prior to screening.
- 20. Any topical treatment for pain including use of:
 - 1. over-the-counter capsaicin on extremities within 12 weeks of screening,
 - 2. and/or Qutenza within 24 weeks of screening,
 - 3. and/or nonsteroidal anti-inflammatory drugs, menthol, methyl salicylate, local anesthetics within 1 week of screening.
- 21. Implanted active medical device (eg, spinal cord stimulator, intrathecal pump, or peripheral nerve stimulator) for the treatment of pain or any etiology.
- 22. Regional anesthetic block for pain of any etiology within 3 months prior to screening.
- 23. Intake in the 4 weeks preceding screening visit of any medication susceptible to inhibit or induce cytochrome P450 CYP2D6.
- 24. Poor metabolizers of CYP2D6 substrates.
- 25. Treatment with an investigational drug in the previous 4 weeks or greater, according to local requirements
- 26. Any condition that the investigator feels would place the patient at increased risk if the investigational therapy is initiated, such as, but not limited to,

	hyperthyroidism, convulsive disorder, advanced hepatic disease, pylorus, stenosis, or paralytic ileus. 27. The investigator considers the patient unfit for the study as a result of the medical interview, physical examination, or screening investigations, in particular any status or disease making the patient unable to follow instructions. 28. The patient is unable to apply the study drug on feet and/or hands. 29. The patient is an employee of the investigator, study site, sponsor, or contract research organization with direct involvement in the proposed study or other studies under the direction of the investigator, study site, or sponsor, or a family member of the site employee or the Investigator.
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Contacts/Locations

Central Contact Person	
Study Officials	
Location	Rochester, Minnesota, United States, 55905 Facility: Mayo Clinic Erlangen, Germany Facility: Universitätsklinikum Erlangen Hautklinik

IPD Sharing

IPD information	Plan to Share IPD: No
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References

Citations	
Links	
Available IPD/Information	

Document Section

Study Protocol and Statistical Analysis Plan	- Document Label Study Protocol and Statistical Analysis Plan - Date 2022-05-06 - Uploaded 2024-12-31 08:23 - File name Prot_SAP_000.pdf ()
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Study Results

Participant Flow

Recruitment Details	[Not Specified]	
Pre-assignment Details	[Not Specified]	

Arm/Group Title	Sequence A	Sequence B
Arm/Group Description	i)screening and randomization; ii) baseline period (3 weeks); iii) treatment period 1 with placebo(3 weeks); iv) wash-out period (3 weeks); v) treatment period 2 with ATX01(3 weeks); vi) follow-up visit Amitriptyline Hydrochloride: hydrogel bottle of ATX01 15% , 0,5ml (1pump) per hand, 1ml (2 pumps) per feet during 3 weeks Placebo: hydrogel bottle of placebo , 0,5ml (1pump) per hand, 1ml (2 pumps) per feet during 3 weeks	i)screening and randomization; ii) baseline period (3 weeks); iii) treatment period 1 with ATX01(3 weeks); iv) wash-out period (3 weeks); v) treatment period 2 with placebo (3 weeks); vi) follow-up visit Amitriptyline Hydrochloride: hydrogel bottle of ATX01 15% , 0,5ml (1pump) per hand, 1ml (2 pumps) per feet during 3 weeks Placebo: hydrogel bottle of placebo , 0,5ml (1pump) per hand, 1ml (2 pumps) per feet during 3 weeks

Period Title: Overall Study

Baseline Characteristics

Arm/Group Title	Sequence A	Sequence B	Total
Arm/Group Description	i)screening and randomization; ii) baseline period (3 weeks); iii) treatment period 1 with placebo(3 weeks); iv) wash-out period (3 weeks); v) treatment period 2 with ATX01(3 weeks); vi) follow-up visit Amitriptyline Hydrochloride: hydrogel bottle of ATX01 15% , 0,5ml (1pump) per hand, 1ml (2 pumps) per feet during 3 weeks Placebo: hydrogel bottle of placebo , 0,5ml (1pump) per hand, 1ml (2 pumps) per feet during 3 weeks	i)screening and randomization; ii) baseline period (3 weeks); iii) treatment period 1 with ATX01(3 weeks); iv) wash-out period (3 weeks); v) treatment period 2 with placebo (3 weeks); vi) follow-up visit Amitriptyline Hydrochloride: hydrogel bottle of ATX01 15% , 0,5ml (1pump) per hand, 1ml (2 pumps) per feet during 3 weeks Placebo: hydrogel bottle of placebo , 0,5ml (1pump) per hand, 1ml (2 pumps) per feet during 3 weeks	Total of all reporting groups
Overall Number of Baseline Participants	7	6	13
Baseline Analysis Population Description	[Not Specified]		

[Expand all](#) / [Collapse all](#)

Age, Categorical

Measure Type: Count of Participants | Unit of measure: Participants

Sex: Female, Male

Measure Type: Count of Participants | Unit of measure: Participants

Ethnicity (NIH/OMB)

Measure Type: Count of Participants | Unit of measure: Participants

Race (NIH/OMB)

Measure Type: Count of Participants | Unit of measure: Participants

Outcome Measures

[Expand all](#) / [Collapse all](#)

1. Comparison Between Treatments (ATX01 15% vs. Placebo) of Mean Pain Attack Intensity Score

Type: Primary | Time Frame: during the last 7 full days (Day 14 to Day 20) of each Treatment Period

Description	Comparison between Treatments (ATX01 15% vs. Placebo) of mean pain attack intensity score assessed for the final week of each treatment period using an 11-point Numerical Pain Rating Scale (NPRS). Mean pain attack intensity is defined as the sum of the pain intensity score of each pain attack during the last 7 full days (Day 14 to Day 20) of each Treatment Period divided by the total number of erythromelalgia pain attacks during that 7-day period
Time Frame	during the last 7 full days (Day 14 to Day 20) of each Treatment Period



Quality Control Review Comment provided by the National Library of Medicine:

1. The description of the scale or categories does not include sufficient information to understand the results reported.

Analysis Population Description	[Not Specified]	
Arm/Group Title	ATX01 15%	Placebo
Arm/Group Description	All patients were administered ATX01 15%, either before or after being administered placebo (Sequence A/Sequence B)	All patients were administered placebo, either before or after being administered ATX01 15% (sequence A/sequence B)
Overall Number of Participants Analyzed	13	13
Mean (95% Confidence Interval) Unit of Measure: units on a scale	4.47 (3.0 to 7.6)	4.5 (2.8 to 6.3)

Statistical Analysis 1

Statistical Analysis Overview

Comparison Group Selection	ATX01 15%, Placebo
Comments	[Not Specified]

Type of Statistical Test	Superiority
Comments	[Not Specified]
Statistical Test of Hypothesis	
P-Value	=0.09
Comments	[Not Specified]
Method	Mixed Models Analysis
Comments	[Not Specified]

2. Mean Pain Attack Intensity Score for Complete Treatment Period

Type: Secondary | Time Frame: through study completion, an average of 15 weeks

Description	11 point Numeric Pain Rating Scale Scale: 0-10 10 means worst possible pain
Time Frame	through study completion, an average of 15 weeks
Analysis Population Description	[Not Specified]

Outcome Measure Data Not Reported

3. Patient Global Impression of Change (PGI-C) Score

Type: Secondary | Time Frame: through study completion, an average of 15 weeks

Description	Patient Global Impression: Global Change (PGI-C) Scale: 1-7 7 means very much worse	
Time Frame	through study completion, an average of 15 weeks	
Analysis Population Description	[Not Specified]	
Arm/Group Title	ATX01 15%	Placebo
Arm/Group Description	All patients were administered ATX01 15%, either before or after being administered placebo (Sequence A/Sequence B)	All patients were administered placebo, either before or after being administered ATX01 15% (sequence A/sequence B)
Overall Number of Participants Analyzed	13	13
Mean (95% Confidence Interval) Unit of Measure: units on a scale	3.7 (3.1 to 4.3)	3.3 (2.7 to 3.8)

Statistical Analysis 1

Statistical Analysis Overview

Comparison Group Selection	ATX01 15%, Placebo
Comments	[Not Specified]
Type of Statistical Test	Superiority
Comments	[Not Specified]

Statistical Test of Hypothesis

P-Value	=1
Comments	[Not Specified]
Method	Mixed Models Analysis
Comments	[Not Specified]

4. Change in Pain Interference With Daily Life Using the Brief Pain Inventory - Short Form Questionnaire (BPI-SF)

Type: Secondary | Time Frame: through study completion, an average of 15 weeks

Description	Brief Pain Inventory Pain Interference Questionnaire Scale: 0-10 10 means completely interferes
Time Frame	through study completion, an average of 15 weeks
Analysis Population Description	[Not Specified]

Outcome Measure Data Not Reported

5. Change in Quality of Life, Measured Using the EuroQol-5-Dimension (EQ-5D) Instrument

Type: Secondary | Time Frame: through study completion, an average of 15 weeks

Description	EuroQol-5-Dimension scale
Time Frame	through study completion, an average of 15 weeks



Quality Control Review Comment provided by the National Library of Medicine:

1. The description of the scale or categories does not include sufficient information to understand the results reported.

Analysis Population Description	[Not Specified]	
Arm/Group Title	ATX01 15%	Placebo
Arm/Group Description	All patients were administered ATX01 15%, either before or after being administered placebo (Sequence A/Sequence B)	All patients were administered placebo, either before or after being administered ATX01 15% (sequence A/sequence B)
Overall Number of Participants Analyzed	13	13
Mean (95% Confidence Interval) Unit of Measure: units on a scale	0.77 (0.71 to 0.84)	0.77 (0.70 to 0.84)

Statistical Analysis 1

Statistical Analysis Overview

Comparison Group Selection	ATX01 15%, Placebo
Comments	[Not Specified]

Type of Statistical Test	Superiority
Comments	[Not Specified]
Statistical Test of Hypothesis	
P-Value	=0.87
Comments	[Not Specified]
Method	Mixed Models Analysis
Comments	[Not Specified]

6. Change in Beck Depression Inventory (BDI) Score.

Type: Secondary | Time Frame: through study completion, an average of 15 weeks

Description	BDI Scale:0-3 3 means worse
Time Frame	through study completion, an average of 15 weeks
Analysis Population Description	[Not Specified]

Outcome Measure Data Not Reported

7. Maximum Pain Intensity Score, Assessed by 11-point NPRS.

Type: Secondary | Time Frame: through study completion, an average of 15 weeks

Description	11 point Numeric Pain Rating Scale Scale: 0-10 10 means worst possible pain
Time Frame	through study completion, an average of 15 weeks
Analysis Population Description	[Not Specified]

Outcome Measure Data Not Reported

8. Mean Number of Pain Attacks Per Day

Type: Secondary | Time Frame: through study completion, an average of 15 weeks

Description	[Not Specified]	
Time Frame	through study completion, an average of 15 weeks	
Analysis Population Description	[Not Specified]	
Arm/Group Title	ATX01 15%	Placebo
Arm/Group Description	All patients were administered ATX01 15%, either before or after being administered placebo (Sequence A/Sequence B)	All patients were administered placebo, either before or after being administered ATX01 15% (sequence A/sequence B)
Overall Number of Participants Analyzed	13	13
Mean (95% Confidence Interval) Unit of Measure: Number of pain attacks per day	11.33 (9.34 to 13.32)	10.24 (8.24 to 12.21)

Statistical Analysis 1

Statistical Analysis Overview

Comparison Group Selection	ATX01 15%, Placebo
Comments	[Not Specified]
Type of Statistical Test	Superiority
Comments	[Not Specified]

Statistical Test of Hypothesis

P-Value	=.08
Comments	[Not Specified]
Method	Mixed Models Analysis
Comments	[Not Specified]

9. Mean and Maximum Duration of Pain Attacks

Type: Secondary | Time Frame: through study completion, an average of 15 weeks

Description	[Not Specified]
Time Frame	through study completion, an average of 15 weeks
Analysis Population Description	[Not Specified]

Outcome Measure Data Not Reported

10. Mean Frequency of Use of Rescue Measures (e.g., Cooling).

Type: Secondary | Time Frame: through study completion, an average of 15 weeks

Description	[Not Specified]
Time Frame	through study completion, an average of 15 weeks
Analysis Population Description	[Not Specified]

Outcome Measure Data Not Reported

11. Mean Duration of Use of Rescue Measures (e.g., Cooling).

Type: Secondary | Time Frame: through study completion, an average of 15 weeks

Description	[Not Specified]
Time Frame	through study completion, an average of 15 weeks
Analysis Population Description	[Not Specified]

Outcome Measure Data Not Reported

12. Change in Pain Score Before and After Rescue Measures (e.g., Cooling).

Type: Secondary | Time Frame: through study completion, an average of 15 weeks

Description	11 point Numeric Pain Rating Scale Scale: 0-10 10 means worst possible pain
Time Frame	through study completion, an average of 15 weeks
Analysis Population Description	[Not Specified]

Outcome Measure Data Not Reported

Adverse Events

Time Frame	Daily from baseline to follow up
Adverse Event Reporting Description	[Not Specified]

Quality Control Review Comment provided by the National Library of Medicine:

1. There does not appear to be sufficient information to understand the Time Frame.

Arm/Group Title	ATX01 15%	Placebo
Arm/Group Description	All patients were administered ATX01 15%, either before or after being administered placebo (Sequence A/Sequence B)	All patients were administered placebo, either before or after being administered ATX01 15% (sequence A/sequence B)

[Expand all](#) / [Collapse all](#)

All-Cause Mortality

Serious Adverse Events

Other (Not Including Serious) Adverse Events

Limitations and Caveats

Description	[Not Specified]
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More Information

Certain Agreements	Principal investigators are NOT employed by the organization sponsoring the study. The only disclosure restriction on the PI is that the sponsor can review results communications prior to public release and can embargo communications regarding trial results for a period that is less than or equal to 60 days from the time submitted to the sponsor for review. The sponsor cannot require changes to the communication and cannot extend the embargo.
Results Point of Contact	Name/Official Title AlgoTherapeutix

Organization	AlgoTherapeutix
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Email	em@algotx.com